TELMISARTAN

Longest half-life ARB

Teratogenic

CHEMICAL NAME 4′-[(1,4′-dimethyl-2′-propyl[2,6′-bi-1H-benzimidazol]-1′-yl)methyl][1,1′-biphenyl]-2-carboxylic acid

CLASS Angiotensin II receptor blocker (ARB)

TRADE NAMES Micardis · Pritor · Micardis Plus (with HCTZ) · Twynsta (with amlodipine)

THERAPEUTIC USES / INDICATIONS

- Hypertension.
- Cardiovascular risk reduction in high-risk patients unable to take ACE inhibitors (ONTARGET).

ADMINISTRATION

- **PO** once daily with or without food.

DOSAGES

- **Hypertension:** 40 mg once daily (20 mg may suffice); max 80 mg.
- **CV prevention:** 80 mg once daily.
- Mild–moderate hepatic impairment: max 40 mg.

PHARMACOKINETICS

- **Onset:** 1–3 h. **Duration:** >24 h.
- Not CYP-metabolised; glucuronidated.
- Excreted almost entirely in bile/faeces. **t½:** 24 h (longest of ARBs).

NURSING MANAGEMENT

- Monitor BP, K^+, renal function.
- Monitor digoxin levels if co-prescribed.
- Check liver function history.
- Postural BP.

PHARMACODYNAMICS

- **AT1 receptor** blockade → vasodilation, ↓ aldosterone.
- Partial PPAR-γ agonism (possible metabolic benefits).

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Dizziness, back pain.
- Diarrhoea, dyspepsia.
- Sinusitis.

ADVERSE EFFECTS (SERIOUS)

- Hyperkalaemia.
- Renal impairment.
- Hypotension; angioedema (rare).

CONTRAINDICATIONS & CAUTIONS

- Pregnancy.
- Biliary obstruction, severe hepatic impairment.
- Aliskiren in diabetes.

KEY INTERACTIONS

- **Digoxin:** ↑ digoxin levels.
- **K^+ supplements:** hyperkalaemia.
- **NSAIDs, lithium:** as for other ARBs.

PATIENT EDUCATION

- Take once daily.
- Report dizziness or swelling.
- Avoid potassium supplements.
- Avoid pregnancy.

KEY POINTS

- ARB with 24 h half-life.
- CV risk reduction (80 mg).
- Biliary excretion; avoid in cholestasis.
- Raises digoxin.